Darolutamide: high-risk nonmetastatic castration-resistant prostate cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Darolutamide

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take the morning and evening doses with food; swallow tablets whole with water. Take a missed dose when remembered before the next scheduled dose, then return to the normal schedule. Do not take two doses together or double the dose. Confirm any prescribed reduction with your team.
- Repeat blood count, kidney and liver tests on your agreed schedule; blood pressure and electrocardiogram (ECG) as clinically indicated. The NCCP regimen does not give a universal blood-test or scan interval. Monitor more closely with kidney or liver impairment.
- Ask pharmacy to check all medicines, especially cholesterol medicines such as rosuvastatin and enzyme-inducing medicines or herbal products. Report jaundice, dark urine, severe tiredness, chest symptoms or a marked change in wellbeing promptly.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00693 v2 · pTreatment / supportive care · NCCP regimen and NCIS antiemetic guidance

Sources checked 2026-11-14 · SACTCheck v0.84.51 · Regimen-level drafts; prescriber and oncology-pharmacy verification required